

Treatment plan and investigations advised

5 September 2026

For Mr M F Rahaman, to carry to every appointment. This plan has been prepared by a physician in the family from your reports of July and August 2026. Each medicine change below is to be confirmed and prescribed by your treating cardiologist — take this sheet, with the enclosed letter, to Dr Sastry at CARE Banjara Hills.

NAME Mr M F Rahaman	AGE / SEX 63 y / Male	DATE OF BIRTH 10 Feb 1963	PLACE Hyderabad
WEIGHT 81 kg	HEIGHT _____ cm — to measure	LAST BP 150 / 100 mmHg	DIABETES reported none — HbA1c pending

DIAGNOSIS What the reports show

- Atrial cardiomyopathy with heart failure, preserved ejection fraction (HFpEF)**
Cardiac MRI, CARE Banjara Hills, 24 Jul 2026 · EF 64% · thick, stiff left ventricle · enlarged, scarred atria
- Paroxysmal atrial fibrillation and atrial flutter — on a blood thinner**
Diagnosed July 2026 · admission for flutter · Apollo summary 14 Aug 2026
- High blood pressure — not controlled at present**
150/100 mmHg on current tablets · was ~120 systolic on the old combination tablet
- Severe obstructive sleep apnoea — not yet treated**
Sleep study: AHI 42.3 events per hour (severe is above 30)
- Raised cholesterol — on a statin**
Lipid test to be repeated
- Family history that matters for your medicines**
Brugada syndrome (nephew, confirmed) · brothers with fainting episodes · heart attacks around age 60 (grandfather, uncle)

#	MEDICINE	DOSE	M	A	N	HOW TO TAKE	STATUS
CONTINUE AS YOU ARE TAKING NOW							
1	Tab. Metoprolol succinate MET-XL	25 mg	1 M	0 A	0 N	After breakfast. Do not increase this tablet for blood pressure — it is for heart rhythm only.	CONTINUE
2	Tab. Apixaban Eliquis	5 mg	1 M	0 A	1 N	Blood thinner. Never miss a dose. Tell any doctor or dentist you take it.	CONTINUE
3	Tab. Atorvastatin Storvas	10 mg	0 M	0 A	1 N	At night. Dose to be reviewed once the lipid test (Investigation 4) is done.	CONTINUE
ASK THE CARDIOLOGIST TO CHANGE							
4	Tab. Perindopril Coversyl	2 → 4 mg, then 8 mg	0 M	0 A	1 N	At night. Increase to 4 mg; to 8 mg after 2–4 weeks if BP is still high and the kidney test is normal. If a dry cough starts: change to Telmisartan 40 mg (the “40” from your old tablet).	INCREASE
5	Tab. Amlodipine the “5” from your old combination tablet	5 mg	1 M	0 A	0 N	Morning. This is the tablet that kept your BP at 120. It is safe with atrial fibrillation. If your ankles swell, tell the doctor — the dose can be halved.	RESTART
ASK THE CARDIOLOGIST TO START							
6	Tab. Spironolactone	25 mg	1 M	0 A	0 N	Morning. Only after Investigations 1 and 2 are done (potassium, kidney, aldosterone). Repeat the potassium and kidney test 1 week and 4 weeks after starting.	START AFTER TESTS
7	Tab. Dapagliflozin or Empagliflozin 10 mg	10 mg	1 M	0 A	0 N	Morning. For the heart failure — works whether or not you are diabetic. You will pass more urine at first; drink normally. Keep the area clean.	START
8	Auto-PAP machine CPAP for sleep apnoea	nightly	≥ 4 h	aim	6+	The most important item on this page. Treats the blood pressure, the heart rhythm and the heart itself at once. A rental trial (about ₹2,000) can start this week. Bring the machine’s card to your reviews.	START NOW

#	MEDICINE	DOSE	M · A · N	HOW TO TAKE	STATUS
ONLY IF A TEST SHOWS YOU QUALIFY					
9	Tirzepatide weekly injection	—	—	Only if your height (Investigation 11) gives a BMI of 30 or more and the HbA1c (Investigation 3) is known. Treats sleep apnoea, heart failure and weight together. In addition to the machine, never instead of it.	TO DISCUSS

DO NOT TAKE — SHOW THIS TO EVERY DOCTOR

AVOID

Flecainide · Propafenone · any “pill in the pocket” rhythm tablet

Because of the Brugada syndrome in our family, these heart-rhythm tablets can be dangerous for you until a specialist has checked your ECG properly (Investigation 10).

INVESTIGATIONS ADVISED Tests to get done before the cardiology visit

- | | |
|---|---|
| <p>☐ 1 Serum potassium, creatinine, eGFR
Before spironolactone — and again 1 and 4 weeks after starting it</p> <hr/> <p>☐ 2 Aldosterone–renin ratio
Must be drawn before spironolactone is started</p> <hr/> <p>☐ 3 HbA1c and fasting glucose
Confirms you are not diabetic</p> <hr/> <p>☐ 4 Lipid profile, fasting
Decides whether 10 mg of atorvastatin is enough</p> <hr/> <p>☐ 5 TSH (thyroid)
Thyroid trouble can cause atrial fibrillation</p> <hr/> <p>☐ 6 NT-proBNP
Heart-strain blood test</p> <hr/> <p>☐ 7 Serum ferritin and transferrin saturation
Iron — low iron worsens breathlessness and is treatable</p> <hr/> | <p>☐ 8 24-hour ambulatory blood pressure monitor
Confirms the readings and checks your BP at night</p> <hr/> <p>☐ 9 Holter / extended heart-rhythm monitor
Shows how much of the day you are in AF, and whether the rate is controlled when you walk</p> <hr/> <p>☐ 10 12-lead ECG with high leads
Ask for “V1–V2 at the 2nd and 3rd intercostal space” — the Brugada check</p> <hr/> <p>☐ 11 Height, measured on a stadiometer
Not in any report — decides your BMI and whether medicine 9 is an option</p> <hr/> <p>☐ 12 Repeat echocardiogram once the heart rate is steady
As the 23 Jul 2026 echo report itself recommended</p> <hr/> |
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APPOINTMENTS Who to see

- **Dr B. K. S. Sastry** — Heart Failure, CARE Hospitals, Banjara Hills
Take the enclosed letter. He will confirm the medicine changes above.

→ **Dr Soumen Devi Dutta** — Electrophysiology, Apollo Hospitals, Jubilee Hills

To discuss a procedure for the flutter (cavotricuspid isthmus ablation), and rhythm control for the AF — the best window for that closes around July 2027.

→ **Sleep clinic / PAP supplier**

Machine set-up and mask fitting this week; pressure check and usage review at 4 weeks and 12 weeks.

ADVICE Every day

- **Blood pressure** morning and evening, seated, after 5 minutes' rest. Write it in a diary and bring the diary.
- **Weight** every morning. Tell the doctor if it rises by 2 kg or more in a week.
- **Ankle swelling** — note it and mention it. Do not stop any tablet on your own.
- **Salt** low; **alcohol** none. Keep walking daily — to the distance that is comfortable, not beyond.
- **Go to hospital the same day** for fainting or near-fainting, palpitations with dizziness, chest pain, or breathlessness at rest.
- Bring **this sheet and all reports** to every visit.

REVIEW When to check

- **2 weeks** after any tablet change — BP check.
- **1–2 weeks** after each perindopril increase — kidney and potassium test.
- **1 week and 4 weeks** after starting spironolactone — kidney and potassium test.
- **4 weeks and 12 weeks** after starting the machine — usage and BP review.
- **Cardiology follow-up** as Dr Sastry advises.

Prepared with love, and with your reports in front of me. Nothing here replaces your cardiologist's judgement — it is written so that the two of you start the conversation from the same page.



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